

Hook & Core Idea

1st Slide | Speaker: Nile

- By the time this pitch ends, someone in the U.S. will walk into an emergency room for a problem that could have been treated earlier at a community clinic at a fraction of the cost.

(Pause. One beat).

- The clinic existed. The appointment was open.

(Pause).

- The patient just couldn't see it.

(Pause).

- That's the problem HealthConnect is built to solve.

2nd Slide | Speaker: Nile

- HealthConnect is an AI-powered healthcare navigation platform that matches patients with affordable, eligible healthcare in real time.

(Stop. Breathe. Let it land).

- That's the idea. And it works for three reasons.

3 Reasons

3rd Slide | Speaker: Nile

- First, healthcare access is not primarily a supply problem. It's a visibility problem.

(Short pause).

- Second, AI makes real-time matching possible in a way current systems can't.

(Short pause).

- And third, helping people access care earlier reduces cost and risk across the system.

(Short pause).

- My colleague will briefly walk through each

4th Slide | Speaker: Eduardo

- When people delay care, it's rarely because care doesn't exist.

(Gesture lightly toward the slide, don't read it).

- It's because pricing, eligibility, and availability are fragmented or unclear.

(Pause on "emergency.")

- That uncertainty causes people to wait. And when they finally seek care, it's often in emergency settings.

- This isn't anecdotal. Nearly 38% of Americans delay or avoid care due to cost or uncertainty.

(Pause).

- That's not a medical failure. It's an information failure.

(Stop. Let that reframe sit).

5th Slide | Speaker: Eduardo

- Most healthcare navigation tools today are static.
- They rely on directories, call centers, or outdated information.

(Small pause).

- Eligibility, pricing, and availability change constantly.

(Slow down).

- AI enables real-time matching at scale, without adding administrative burden.

(Key line. Slow down).

- This isn't about replacing providers.
- It's about replacing guesswork. So clinical staff can actually focus on treating patients.

(Pause. This is one of your strongest lines).

6th Slide | Speaker: Eduardo

- Delayed care doesn't save money.
- It shifts cost downstream, often into emergency rooms.

(Pause).

- Those costs don't disappear. They shift to hospitals, public systems, and ultimately taxpayers.

(Pause).

- Late-stage treatment is significantly more expensive than early intervention.
- Improving access earlier creates system-wide efficiency gains.

(Do not overexplain. This slide closes the logic loop).

How does it work and ROI for providers

7th Slide | Speaker: Nile

- So how does HealthConnect actually do this?
- From the patient's perspective, HealthConnect is intentionally simple.
- Patients enter basic information...
- ...they are matched to care they qualify for...

- ...and then, they can book an appointment in just a few steps.

(Gesture across the three wireframes once. Do not narrate UI).

- While it's simple for the user, our engine is doing the heavy lifting—pulling in live data and verifying eligibility to point patients toward the right clinics."

- The system is designed for HIPAA compliance and data security from the start.

(Say this calmly. No defensiveness).

8th Slide | Speaker: Eduardo

- Last but not least, it's also important to note that providers already pay for visibility and booking systems today.

- HealthConnect fits into an existing spend, but delivers it with eligibility and real-time matching, creating operational ROI for providers.

(Pause. This sentence matters).

- We reduce administrative overhead through automated screening.
- We lower no-show rates by matching eligible patients to available appointments.
- And we improve patient capacity without increasing staffing.

(Do not rush).

- That's why providers have a reason to partner with us.

Closing Slide

9th Slide | Speaker: Nile

- Our vision is to become the universal navigation tool for affordable healthcare.

(Pause. Let the ambition breathe).

- The next step is to launch a pilot with community clinics to see firsthand how our matching engine improves patient health and a clear ROI for providers.

- **Eduardo closes:** Let's bridge the gap between those who need care and those who provide it, and make healthcare visibility a right, not a privilege.

Top Judge Questions

1. “Why hasn’t this been done already?”

Answer (tight): “Pieces of this exist, but they’re fragmented and static. What’s changed is the ability to aggregate eligibility, pricing, and availability in real time. That’s what makes this viable now.”

2. “Who pays for this?”

Answer: “Primarily providers, because they see direct operational ROI. Access to the patients will be free by design. We are not introducing a new cost category, we’re improving how existing visibility and booking spend is used.”

3. “How do you get clinics to adopt this?”

Answer: “Providers already pay for visibility and booking tools. HealthConnect fits into existing spend, but reduces waste by filtering for eligibility and availability before patients ever arrive.”

4. “Isn’t this just Google/Yelp for healthcare?”

Answer: “Most existing solutions rely on static directories or manual updates. HealthConnect continuously updates eligibility, pricing, and availability in real time — that’s the core differentiation.”

5. “How do you handle HIPAA and patient data?”

Answer: “We minimize data collection, use role-based access, and design the system so sensitive information is only used when necessary. HIPAA compliance is a design constraint, not an afterthought.”

6. “What does success look like in the pilot?”

Answer: “We’re measuring three things: reduced no-show rates, faster access to care, and improved clinic utilization — all of which translate directly into provider ROI and better patient outcomes.”

7. “What’s the biggest risk?”

Answer: “Data quality and integration speed. That’s why we’re starting with a focused pilot and verified clinics before scaling.”

8. “How do they find you?”:

Answer: “We don’t expect every patient to find us on the App Store on day one. Our go-to-market is partnership-first. We reach patients through trusted gatekeepers like social workers and 211 operators, who are already helping people find care.”

9. “How do you get the data?”

Answer: “We start with publicly available sources and clinic partnerships, then validate availability and eligibility through structured data feeds. The pilot phase is specifically about validating and refining that data layer.”

10. “What’s your initial target market?”

Answer: “Uninsured and underinsured adults seeking primary and preventive care, and the community clinics that already serve them.”

11. “What’s the hardest part of execution?”

Answer: “Data consistency across providers. That’s why we’re starting with a pilot and a narrow scope before scaling.”

12. “How do you scale this?”

Answer: “Once validated, the model scales linearly with clinics and regions. The matching logic doesn’t change, only coverage does.”

12. “What happens if clinics don’t cooperate?”

Answer: “That’s exactly why provider ROI is central to the model. If it doesn’t reduce admin burden or no-shows, clinics won’t adopt it—and the pilot is designed to test that.”

13. “What’s the ethical concern here?”

Answer: “We don’t diagnose or recommend treatment. We help people find care they’re already eligible for. The ethical risk is delaying care due to lack of information.”
